United Healthcare

UnitedHealthcare Commercial

Medical Policy Update Bulletin: March 2026

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Updated

Policy Title	Effective Date	Summary of Changes
Category III Codes	Apr. 1, 2026	**Applicable Codes** • Removed CPT codes 0660T, 0795T, 0796T, 0797T, 0798T, 0799T, 0800T, 0801T, 0802T, and 0803T
Lower Extremity Endovascular Procedures	Mar. 1, 2026	**Medical Records Documentation Used for Reviews** • Updated list of Medical Records Documentation Used for Reviews; replaced: ○ "Relevant history and physical to include member symptoms and pertinent findings due ischemia" with "relevant history and physical to include member symptoms and pertinent findings due ischemia *with specific description of location, aggravating, and alleviating factors of limb pain*" ○ "Treatments tried, failed, and/or contraindicated; include the dates, duration, and reason for discontinuation (e.g., *exercise program* pharmacologic therapy, and *smoking cessation*)" with "treatments tried, failed, and/or contraindicated; include the dates, duration, and *monitoring protocol for exercise therapy specific to peripheral vascular disease (PVD), the method, dates, and duration of attempted smoking cessation trial, and* reason for discontinuation (e.g., pharmacologic therapy)" ○ "Documentation of ischemic peripheral artery disease including Ankle-Brachial Index (ABI)" with "documentation of ischemic peripheral artery disease including Ankle-Brachial Index (ABI) *or toe-brachial index (TBI) if non-compressible*"
Surgery of the Hip	Mar. 1, 2026	**Definitions** • Updated definition of "Radiographic Findings of Osteoarthritis" **Supporting Information** • Updated *References* section to reflect the most current information
Surgery of the Knee	Mar. 1, 2026	**Definitions** • Added definition of: ○ Disabling Pain ○ Functional Disability ○ Outerbridge Grades ○ Radiographic Findings of Osteoarthritis ○ Western Ontario and McMaster Universities Arthritis Index (WOMAC) **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Lower Extremity Endovascular Procedures	May 1, 2026	**Coverage Rationale** • Added language to indicate intravascular lithotripsy for treating lower extremity ischemia is unproven and not medically	**Note**: This policy does not apply to upper extremities. **Endovascular revascularization procedures (e.g., stents, angioplasty, and/or atherectomy) are proven and medically necessary for treating non–limb-threatening lower extremity ischemia in individuals with**

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Lower Extremity Endovascular Procedures (continued)	May 1, 2026	necessary due to insufficient evidence of efficacy **Applicable Codes** • Added CPT codes 37262 and 37279 **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, and *References* sections to reflect the most current information	**Claudication due to atherosclerotic disease of the aortoiliac and/or femoropopliteal arteries when all the following criteria are met:** • Impaired ability to work and/or perform activities of daily living; and • **All** the following conservative therapies have been tried and failed: ○ At least 12 weeks of Supervised Exercise Therapy or a Structured Community-Based Exercise Program; and ○ Pharmacological therapy (e.g., lipid-lowering therapy, antihypertensive therapy, antiplatelet therapy, and/or anticoagulants); and ○ Smoking cessation, if applicable and • Ischemic peripheral artery disease with Ankle-Brachial Index of ≤ 0.90; and • Imaging results of the target vessel (e.g., duplex ultrasound, computed tomography angiography, magnetic resonance angiography, or digital subtraction angiography) show anatomical location and a moderate-severe stenosis (50% or greater); if duplex ultrasound does not demonstrate a stenosis of 50% or greater, another imaging modality will be necessary to demonstrate the extent of stenosis **Retreatment of a previously treated vessel due to in-stent restenosis is proven and medically necessary for treating non–limb-threatening lower extremity ischemia in individuals with Claudication due to atherosclerotic disease of the aortoiliac and/or femoropopliteal arteries when all the following criteria are met:** • Recurrent symptoms; and • Impaired ability to work and/or perform activities of daily living; and • Imaging results of the previously treated vessel (e.g., duplex ultrasound, computed tomography angiography, magnetic resonance angiography, or digital subtraction angiography) show anatomical location and a moderate-severe stenosis (50% or greater); if duplex ultrasound does not demonstrate a stenosis of 50% or greater, another imaging modality will be necessary to demonstrate the extent of stenosis **Endovascular revascularization procedures (e.g., stents, angioplasty, and/or atherectomy) are proven and medically necessary for treating**

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Lower Extremity Endovascular Procedures (continued)	May 1, 2026		**Chronic Limb-Threatening Ischemia, with the diagnoses listed under *Applicable Codes* section of the policy.** **Endovascular revascularization procedures (e.g., stents, angioplasty, and/or atherectomy) for treating lower extremity ischemia are unproven and not medically necessary in the following circumstances due to insufficient evidence of efficacy:** • Interventions performed for non–limb-threatening infrapopliteal (e.g., anterior tibial, posterior tibial, or peroneal) artery disease • Individual is asymptomatic • To prevent the progression of Claudication to Chronic Limb-Threatening Ischemia • Transluminal peripheral atherectomy of the iliac artery • Treatment of a nonviable limb **Endovenous femoropopliteal bypass using a stent graft is unproven and not medically necessary for treating peripheral artery disease due to insufficient evidence of efficacy.** **Intravascular lithotripsy for treating lower extremity ischemia is unproven and not medically necessary due to insufficient evidence of efficacy.**
Outpatient Cardiology Procedures for EviCore Arrangement (for Oxford Only)	Apr. 1, 2026	**Applicable Codes** • Revised list of CPT codes requiring prior authorization through EviCore healthcare; added 0042T	Oxford has engaged EviCore, by Evernorth, to perform initial reviews of requests for prior authorization that may include a site of service review (Oxford continues to be responsible for decisions to limit or deny coverage and for appeals). Refer to the Medical Policy titled Magnetic Resonance Imaging (MRI) and Computed Tomography (CT) Scan – Site of Service. All prior authorization requests are handled by EviCore. To obtain prior authorization for a cardiology procedure, contact EviCore via one of the three options below: • Providers can call 1-877-PRE-AUTH (1-877-773-2884); or • Providers can log onto the Prior Authorization and Notification App EviCore has established correct coding and evidence-based criteria to determine the medical necessity and appropriate billing of cardiology services. These criterions have been carefully researched and are

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Outpatient Cardiology Procedures for EviCore Arrangement (for Oxford Only) (continued)	Apr. 1, 2026		continually updated in order to be consistent with the most current evidence-based criteria. The cardiology evidence-based criteria and management criteria are available on the EviCore website using the Prior Authorization and Notification App. Treating Cardiology providers may be asked to submit a clinical submission form. The following information/documentation may be required: • Copies of office notes and treatment planning documents • Results of key diagnostic studies and/or office notes In conjunction with board certified cardiologists and radiologists, EviCore staff will evaluate the submitted treatment and billing plans. Providers will be informed, in writing, as to which services have been approved for payment. Where provided by state regulations, a board-certified cardiologist will be available to discuss the payment decision with the treating provider. **Payment by Specialty and Accreditation Requirements** Participating providers will be reimbursed for radiology and cardiology services rendered in the office or in an outpatient setting. The following is a list of requirements by cardiac service that are payable to participating providers based on their specialty as well as their accreditation status. Prior authorization is required. Oxford has engaged EviCore to manage the accreditation process for our provider network. Accreditations should be submitted directly to the EviCore website. To ensure prompt handling of the accreditation, ensure that all applicable facility and physician information is included. This policy assumes board certification by an American Board of Medical Specialties (ABMS) recognized in the provider specialty listed below. **Note**: Hospitals are currently excluded from the privileging and accreditation requirements below. **Payment by Specialty requirements for participating providers to perform cardiac services:**

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Outpatient Cardiology Procedures for EviCore Arrangement (for Oxford Only) (continued)	Apr. 1, 2026		

Modality	Specialty
Nuclear Medicine, Cardiac CT Scan, Cardiac PET, and Cardiac MRI	• Radiologist • Radiology center/facility • Certified cardiologist • Cardiovascular disease specialists
Diagnostic Cardiac Heart Catheterizations	• Cardiovascular disease • Cardiology group • Pediatric cardiology • Cardiology • Clinical cardiac electro physician • Cardiac electrophysiology

Accreditation requirements for participating providers to perform cardiac services:

Modality	Certification Required
Cardiac CT Scan, Cardiac PET, Cardiac MRI, and Nuclear Medicine	ACR (American College of Radiology); IAC (Intersocietal Accreditation Commission); RadSite; or The Joint Commission (TJC)

Payment Guidelines

- The notification/authorization number is valid for 45 calendar days. It is specific to the advanced outpatient imaging procedure requested, to be performed one time, for one date of service within the 45-day period.
- Current Procedural Terminology (CPT) codes that are not subject to TC/PC component may be reimbursed to both the physician and facility when billed for the same date of service (DOS).
- ECG, diagnostic studies, and injection procedures must be billed in conjunction with an authorized cardiac catheterization code in order to be reimbursed on the same date of service. When billed in conjunction with an authorized cardiac catheterization, no separate authorization will be required in addition to the catheterization code for these services.

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Outpatient Radiology Procedures for EviCore Arrangement (for Oxford only)	Apr. 1, 2026	**Notice of Revision**: The following summary of changes has been modified. Revisions to the previous policy update announcement are outlined in red below. Please take note of the amended updates to be applied on **Apr. 1, 2026**. **Applicable Codes** • Revised list of CPT codes requiring prior authorization through EviCore healthcare; added ~~0042T, 0742T,~~ 0865T, 0866T, 70472, and 70473	Oxford has engaged EviCore, by Evernorth, to perform initial reviews of requests for prior authorization and medical necessity reviews that may include a site of service review. (Oxford continues to be responsible for decisions to limit or deny coverage and for appeals). Refer to the Medical Policy titled Magnetic Resonance Imaging (MRI) and Computed Tomography (CT) Scan – Site of Service. All prior authorization requests are handled by EviCore. To prior authorize a radiology procedure, contact EviCore via one of the two options listed below: • Providers can call EviCore at 1-877-PRE-AUTH (1-877-773-2884); or • Providers can log onto the Prior Authorization and Notification App **Note**: It is Oxford's policy not to accept prior authorization requests from persons or entities other than referring physicians. The notification/authorization number is valid for 45 calendar days. It is specific to the advanced outpatient imaging procedure requested, to be performed one time, for one date of service within the 45-day period. EviCore has established an infrastructure to support the review, development, and implementation of comprehensive outpatient imaging criteria. The radiology evidence-based guidelines and management criteria are available on the EviCore web site using the Prior Authorization and Notification App. **Accreditation Requirements for Participating Providers** **Note**: Hospitals are currently excluded from the accreditation requirements listed below. All MRI, PET, CT, nuclear medicine, and ultrasound studies must be performed on an American College of Radiology (ACR), American Institute of Ultrasound in Medicine (AIUM), Intersocietal Accreditation Commission (IAC), RadSite, or The Joint Commission (TJC) accredited unit or at accredited facilities. Refer to the Administrative Policy titled Accreditation Requirements for Radiology Services.

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Outpatient Radiology Procedures for EviCore Arrangement (for Oxford only) (continued)	Apr. 1, 2026		Oxford has engaged EviCore to manage the accreditation process for our provider network. Accreditations should be submitted directly to the EviCore website. To ensure prompt handling of the accreditation, ensure that all applicable facility and physician information is included. The Oxford Radiology Prior Notification/Authorization Crosswalk Table contains a list of CPT® codes that are interchangeable for prior authorization. If a provider obtains prior authorization for a procedure that corresponds with the Crosswalk Table, then the substitution is appropriate.

Preventive Care Services	Apr. 1, 2026	**Applicable Codes**
		Preventive Care Services
		Cervical Cancer Screening
		• Added ICD-10 diagnosis codes Z01.42, Z12.72, and Z12.79
		Osteoporosis Screening
		• Revised service description:
		  ○ Removed Jun. 2018 USPSTF "B" rating
		  ○ Added Jan. 2025 USPSTF "B" rating to indicate the USPSTF recommends screening for osteoporosis to prevent osteoporotic fractures in:
		  ▪ Women 65 years or older
		  ▪ Postmenopausal women younger than 65 years who are at increased risk for an osteoporotic fracture as estimated by clinical risk assessment
		• Removed CPT/HCPCS codes 76977, 77081, and G0130
			Refer to the policy for complete details.

Spinal Fusion and Decompression	Apr. 1, 2026	**Related Policies** • Added reference link to the Medical Policy titled *Interspinous*	**Spinal procedures for the treatment of spine pain are proven and medically necessary in certain circumstances.** For medical necessity clinical coverage criteria, refer to the InterQual® CP: Procedures: • Decompression +/- Fusion, Cervical

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Spinal Fusion and Decompression (continued)	Apr. 1, 2026	*Fusion and Decompression Devices* **Coverage Rationale** • Revised list of unproven and not medically necessary indications: ○ Added "vertebral joint implants that replace the disc and facet joints (e.g., MOTUS) for the treatment of spine pain" ○ Replaced: ▪ "Dynamic Stabilization systems for the treatment *of degenerative Spondylolisthesis*" with "Dynamic Stabilization systems for the treatment *of spine pain*" ▪ "Isolated Facet Joint Fusion, with or without instrumentation" with "Isolated Facet Joint Fusion, with or without instrumentation, *for the treatment of spine pain*" ▪ "*Total* Facet Joint *Arthroplasty*" with "Facet Joint *Replacement for treatment of spine pain*" **Definitions** • Added definition of "Facet Joint Replacement" • Removed definition of: ○ Disabling Symptoms ○ Lumbar Spinal Stenosis (LSS) ○ Progressive ○ Radicular Pain	• Decompression +/- Fusion, Lumbar • Decompression +/- Fusion, Thoracic • Fusion, Cervical Spine • Fusion, Lumbar Spine • Fusion, Thoracic Spine • Scoliosis or Kyphosis Surgery • Scoliosis or Kyphosis Surgery (Pediatric) Click here to view the InterQual® criteria. **Dividing treatment of symptomatic, multisite spinal pathology via anterior or posterior approach into serial or Staged Multiple Sessions when one session can address all sites is unproven and not medically necessary due to insufficient evidence of safety and efficacy.** **The following procedures for the treatment of spine pain are unproven and not medically necessary due to insufficient evidence of efficacy:** • Dynamic Stabilization systems • Facet Joint Replacement • Isolated Facet Joint Fusion, with or without instrumentation • Vertebral joint implants that replace the disc and facet joints (e.g., MOTUS)

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spinal Fusion and Decompression (continued)	Apr. 1, 2026	○ Spinal Fusion ○ Spondylolisthesis ○ Spondylolysis ○ Total Facet Arthroplasty ○ Unremitting • Updated definition of: ○ Dynamic Stabilization ○ Isolated Facet Joint Fusion ○ Staged Multiple Sessions **Applicable Codes** • Removed CPT codes 63170, 63172, 63173, 63185, 63190, 63191, 63197, 63200, 63250, 63251, 63252, and 63265 **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	
Treatment of Temporomandibular Joint Disorders	Mar. 1, 2026	**Notice of Revision**: The following summary of changes has been modified. Revisions to the previous policy update announcement are outlined in red below. Please take note of the amended updates to be applied on **Mar. 1, 2026**. **Coverage Rationale** • Revised list of unproven and not medically necessary services; removed: ○ Craniosacral manipulation/therapy ○ ~~Jaw mobility mechanical stretching devices (e.g., Therabite Jaw Motion~~	**The following nonsurgical services are proven and medically necessary for treating disorders of the temporomandibular joint (TMJ):** • Arthrocentesis • Intra-articular injections of corticosteroids • Trigger point injections • Physical therapy • Occlusal splint (stabilization and repositioning splints) **The following TMJ surgical services are proven and medically necessary in certain circumstances.** For medical necessity clinical coverage criteria, refer to the: • InterQual® CP: Procedures: ○ Arthroscopy, Temporomandibular Joint (TMJ) ○ Discectomy, Temporomandibular Joint (TMJ) ○ Reconstruction, Temporomandibular Joint (TMJ) • InterQual® Client Defined, CP: Procedures, Arthroplasty, Temporomandibular Joint (TMJ) (Custom) – UHG

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Treatment of Temporomandibular Joint Disorders (continued)	Mar. 1, 2026	~~Rehabilitation System®, Jaw Dynasplint® System)~~ ○ ~~Multiple occlusal splints (i.e., daytime and nighttime splints, maxillary and mandibular splints)~~ **Applicable Codes** • Removed CPT codes 21089 and 21499 **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, and *References* sections to reflect the most current information	Click here to view the InterQual® criteria. **The following services are unproven and not medically necessary for treating disorders of the TMJ due to insufficient evidence of efficacy (this list is not all inclusive):** • Biofeedback • Jaw mobility mechanical stretching devices (e.g., TheraBite Jaw Motion Rehabilitation System®, Jaw Dynasplint® System) • Multiple occlusal splints (e.g., daytime and nighttime splints, maxillary and mandibular splints) • Epigenetic appliances [e.g., Homeoblock™, DNA® (Daytime/Nighttime appliance), Advanced Lightwire Functional appliance] For information regarding intra-articular injections of sodium hyaluronate for TMJ disorders, refer to the Medical Benefit Drug Policy titled Sodium Hyaluronate. For information regarding botulinum toxin injections for TMJ disorders, refer to the Medical Benefit Drug Policy titled Botulinum Toxins A and B.
Vertebral Body Tethering for Scoliosis	Apr. 1, 2026	**Coverage Rationale** • Revised language to indicate: ○ Vertebral body tethering (VBT) surgery may be medically necessary for idiopathic scoliosis when all the following criteria are met: ▪ The individual meets all the following clinical criteria: – There is physician documentation to establish failed conservative management (e.g., bracing, observation, or physical therapy) prior to the initial	**Vertebral body tethering (VBT) surgery may be medically necessary for idiopathic scoliosis when all the following criteria are met:** • The individual meets all the following clinical criteria: ○ There is physician documentation to establish failed conservative management (e.g., bracing, observation, or physical therapy) prior to the initial procedure with curvature progression to at least 45 degrees; and ○ The Cobb Angle of the major coronal curve is 45 to 65 degrees for the single curve planned for surgery and none of the spinal curves present are greater than 65 degrees; and ○ Skeletal immaturity is defined by the Sanders Maturity Score of 2 to 5; and ○ The Cobb Angle decreases in magnitude to 30 degrees or less on bending films; and ○ Osseous structure is dimensionally adequate to accommodate screw fixation; and ○ The VBT instrumentation does not extend above T4 or below L4 and

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Vertebral Body Tethering for Scoliosis (continued)	Apr. 1, 2026	procedure with curvature progression to at least 45 degrees – The Cobb Angle of the major coronal curve is 45 to 65 degrees for the single curve planned for surgery and none of the spinal curves present are greater than 65 degrees – Skeletal immaturity is defined by the Sanders Maturity Score of 2 to 5 – The Cobb Angle decreases in magnitude to 30 degrees or less on bending films – Osseous structure is dimensionally adequate to accommodate screw fixation – The VBT instrumentation does not extend above T4 or below L4 ▪ The facility where the surgical procedure will be performed has all the following: – An established, on-site surgical pediatric scoliosis program	• The facility where the surgical procedure will be performed has all the following: ○ An established, on-site surgical pediatric scoliosis program; and ○ Inpatient pediatric physical therapy is available for post-operative training; and ○ Intraoperative advanced imaging capability; and ○ A pediatric anesthesiologist on staff; and ○ A pediatric intensive care unit and • The surgery will be performed by the pediatric orthopedic spine surgeon with experience in scoliosis and surgical procedures such as VBT and who has determined that the procedure is appropriate for the individual; and • The pediatric orthopedic spine surgeon is listed as an investigator on a prospective research study being performed at the pediatric spine center that has an approved Institutional Review Board protocol that is actively recruiting participants for VBT utilizing The Tether™ Vertebral Body Tethering System, which has FDA approval under a Humanitarian Device Exemption and for which the member is a study cohort candidate; and • The individual and family have engaged in a Shared Decision-Making conversation with the pediatric orthopedic spine surgeon **Revision surgery for VBT may be medically necessary when one or more of the following are present:** • Tether breakage or other hardware failure; or • Under- or over-correction of curves; or • Removal of tether and/or anchor screws for surgical complication (e.g., impingement on vital organs, infection, intractable pain) **VBT surgery is not medically necessary when the above criteria are not met.** Refer to the *U.S. Food and Drug Administration (FDA)* section of the policy for information regarding FDA labeling and Humanitarian Device Exemption for VBT.

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Vertebral Body Tethering for Scoliosis (continued)	Apr. 1, 2026	– Inpatient pediatric physical therapy is available for post-operative training – Intraoperative advanced imaging capability – A pediatric anesthesiologist on staff – A pediatric intensive care unit ▪ The surgery will be performed by the pediatric orthopedic spine surgeon with experience in scoliosis and surgical procedures such as VBT and who has determined that the procedure is appropriate for the individual ▪ The pediatric orthopedic spine surgeon is listed as an investigator on a prospective research study being performed at the pediatric spine center that has an approved Institutional Review Board protocol that is actively recruiting participants for VBT utilizing The Tether™ Vertebral Body Tethering System, which has FDA approval under a	

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Vertebral Body Tethering for Scoliosis (continued)	Apr. 1, 2026	Humanitarian Device Exemption and for which the member is a study cohort candidate ▪ The individual and family have engaged in a Shared Decision-Making conversation with the pediatric orthopedic spine surgeon ○ Revision surgery for VBT may be medically necessary when one or more of the following are present: ▪ Tether breakage or other hardware failure ▪ Under- or over-correction of curves ▪ Removal of tether and/or anchor screws for surgical complication (e.g., impingement on vital organs, infection, intractable pain) ○ VBT surgery is not medically necessary when the above criteria are not met ○ Refer to the *U.S. Food and Drug Administration (FDA)* section [of the policy] for information regarding FDA labeling and Humanitarian Device Exemption for VBT **Medical Records Documentation Used for Reviews** ● Added language to indicate:	

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Vertebral Body Tethering for Scoliosis (continued)	Apr. 1, 2026	○ Benefit coverage for health services is determined by the member specific benefit plan document and applicable laws that may require coverage for a specific service ○ Medical records documentation may be required to assess whether the member meets the clinical criteria for coverage but does not guarantee coverage of the service requested; refer to the guidelines titled Medical Records Documentation Used for Reviews **Definitions** • Added definition of: ○ Cobb Angle ○ Lenke Classification System ○ Institutional Review Board (IRB) ○ Sanders Skeletal Maturity Staging System ○ Shared Decision-Making **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	

Updated

Policy Title	Effective Date	Summary of Changes
Evkeeza® (Evinacumab-Dgnb)	Apr. 1, 2026	**Applicable Codes** • Removed ICD-10 diagnosis codes E78.011, E78.019, and Z83.42 **Supporting Information** • Updated *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information
Respiratory Interleukins (Cinqair®, Fasenra®, & Nucala®)	Apr. 1, 2026	**Coverage Rationale** • Added language to indicate Exdensur™ (depemokimab-ulaa) has been added to the Review at Launch program and some members may not be eligible for coverage of this medication at this time; refer to the Medical Benefit Drug Policy titled *Review at Launch for New to Market Medications* for additional details

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Complement C5 Inhibitors	Apr. 1, 2026	**Title Change** • Previously titled *Complement Inhibitors* **Coverage Rationale** • Revised coverage criteria for: ***Atypical Hemolytic Uremic Syndrome (aHUS)*** Continuation of Therapy ○ Added criterion requiring one of the following: ▪ Patient is on Soliris and meets the criteria in the *Preferred Product Criteria* section [of the policy] ▪ Patient is on Bkemv, Epysqli, or Ultomiris ○ Removed criterion requiring the patient has previously been treated with the requested product ***Paroxysmal Nocturnal Hemoglobinuria (PNH)*** Continuation of Therapy ○ Added criterion requiring one of the following:	This policy refers only to the following complement inhibitor drug products: • Bkemv (eculizumab-aeeb) • Epysqli (eculizumab-aagh) • PiaSky (crovalimab-akkz) • Soliris (eculizumab) • Ultomiris (ravulizumab-cwvz) Refer to the policy for complete details.

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Complement C5 Inhibitors (continued)	Apr. 1, 2026	▪ Patient is on Soliris and meets the criteria in the *Preferred Product Criteria* section [of the policy] ▪ Patient is on Bkemv, Epysqli, PiaSky, or Ultomiris ○ Removed criterion requiring the patient has previously been treated with the requested product ***Generalized Myasthenia Gravis in Patients who are Anti-Acetylcholine Receptor (AChR) Antibody Positive*** Initial Therapy ○ Added criterion requiring the patient is not receiving the requested product in combination with the following for treatment of the same indication: ▪ B-cell depletion therapy [e.g., Uplizna (inebilizumab)] ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) Continuation of Therapy ○ Added criterion requiring: ▪ The patient is not receiving the requested product in combination with the following for treatment of the same indication: – B-cell depletion therapy [e.g., Uplizna	

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Complement C5 Inhibitors (continued)	Apr. 1, 2026	(inebilizumab)] – An immune globulin (e.g., Hizentra, Privigen, Gammagard) ▪ One of the following: – Patient is on Soliris and meets the criteria in the *Preferred Product Criteria* section [of the policy] – Patient is on Bkemv, Epysqli, or Ultomiris ○ Removed criterion requiring the patient has previously been treated with the requested product ***Neuromyelitis Optica Spectrum Disorder (NMOSD)*** Initial Therapy ○ Removed criterion requiring one of the following: ▪ History of at least two relapses during the previous 12 months prior to initiating the requested product ▪ History of at least three relapses during the previous 24 months, at least one relapse occurring within the past 12 months prior to initiating the requested product Continuation of Therapy ○ Added criterion requiring one of the following:	

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Complement C5 Inhibitors (continued)	Apr. 1, 2026	▪ Patient is on Soliris and meets the criteria in the *Preferred Product Criteria* section [of the policy] ▪ Patient is on Bkemv, Epysqli, or Ultomiris ○ Removed criterion requiring the patient has previously been treated with the requested product	
Denosumab	Apr. 1, 2026	**Coverage Rationale** • Revised preferred product criteria; replaced criterion requiring: ○ "[The patient has a] history of trial of adequate dose and duration of Prolia ***or*** Stoboclo, resulting in minimal clinical response" with "[the patient has a] history of trial of adequate dose and duration of Prolia ***and*** Stoboclo, resulting in minimal clinical response" ○ "The physician attests that, in their clinical opinion, the clinical response would be expected to be superior with Jubbonti or other non-preferred denosumab product, than experienced with Prolia ***or*** Stoboclo" with "the physician attests that, in their clinical opinion, the clinical response would be expected to be superior with Jubbonti or other non-preferred denosumab product, than experienced with Prolia ***and*** Stoboclo"	**Bildyos® (denosumab-nxxp), Bosaya™ (denosumab-kyqq), Conexxence® (denosumab-bnht), Enoby™ (denosumab-qbde), and Ospomyv™ (denosumab-dssb) have been added to the Review at Launch program. Some members may not be eligible for coverage of this medication at this time. Refer to the Medical Benefit Drug Policy titled Review at Launch for New to Market Medications for additional details.** This policy refers to the following denosumab products for non-oncology conditions: • Jubbonti® (denosumab-bbdz) • Prolia® (denosumab) • Stoboclo® (denosumab-bmwo) • Any FDA-approved denosumab product not listed here* *Any U.S. Food and Drug Administration approved denosumab product not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare. For oncology indications, refer to the Medical Benefit Drug Policy titled Oncology Medication Clinical Coverage for updated information based upon the National Comprehensive Cancer Network (NCCN) Drugs & Biologics Compendium® (NCCN Compendium®). **Jubbonti® is typically excluded from coverage. Coverage reviews may be in place if required by law or the benefit plan.** Refer to the Medical Benefit Drug Policy titled Medical Benefit Therapeutic Equivalent

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Denosumab (continued)	Apr. 1, 2026	○ "[The patient has a] history of intolerance, contraindication, or adverse event to Prolia ***or*** Stoboclo" with "[the patient has a] history of intolerance, contraindication, or adverse event to Prolia ***and*** Stoboclo"	Medications – Excluded Drugs and the corresponding excluded drug list with preferred alternatives. **Preferred Product** ***Medical Necessity Plans*** Prolia and Stoboclo are the preferred denosumab products. Coverage will be provided for Prolia and Stoboclo contingent on the coverage criteria in the Diagnosis-Specific Criteria section. Coverage for Jubbonti or other non-preferred denosumab products will be provided contingent on the criteria in this Preferred Product Criteria section and the coverage criteria in the Diagnosis-Specific Criteria section. In order to continue coverage, members already on Jubbonti or other non-preferred denosumab products will be required to change therapy to Prolia or Stoboclo unless they meet the criteria in this section. **Preferred Product Criteria** **Treatment with Jubbonti or other non-preferred denosumab products is medically necessary for the indications specified in the policy when one of the following is met:** • **Both** of the following: ○ History of a trial of adequate dose and duration of Prolia and Stoboclo, resulting in minimal clinical response; **and** ○ Physician attests that, in their clinical opinion, the clinical response would be expected to be superior with Jubbonti or other non-preferred denosumab product, than experienced with Prolia and Stoboclo **or** • **Both** of the following: ○ History of intolerance, contraindication, or adverse event to Prolia and Stoboclo; **and** ○ Physician attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with Jubbonti or other non-preferred denosumab product

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Denosumab (continued)	Apr. 1, 2026		***Non-Medical Necessity Plans***
			Any denosumab product is to be approved contingent on the coverage criteria in the Diagnosis-Specific Criteria section.
			
			Diagnosis-Specific Criteria
			"Denosumab" will be used to refer to all denosumab products.
			
			Denosumab is proven for the treatment of postmenopausal patients with osteoporosis or to increase bone mass in patients with osteoporosis at high risk for fracture when all of the following criteria are met:
			• **Initial Therapy**
			  ○ Diagnosis of osteoporosis; **and**
			  ○ Patient is at high risk for fracture (e.g., history of osteoporotic fracture, multiple risk factors for fracture, patients who have failed or are intolerant to other available osteoporosis therapy); **and**
			  ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and**
			  ○ Authorization is for no more than 12 months
			• **Reauthorization/Continuation of Care Criteria**
			  ○ Documentation of positive clinical response to therapy; **and**
			  ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and**
			  ○ Authorization is for no more than 12 months
			
			Denosumab is proven to treat glucocorticoid-induced osteoporosis in patients at high risk for fracture when all of the following criteria are met:
			• **Initial Therapy**
			  ○ Diagnosis of glucocorticoid-induced osteoporosis; **and**
			  ○ Patient is at high risk for fracture (e.g., history of osteoporotic fracture, multiple risk factors for fracture, patients who have failed or are intolerant to other available osteoporosis therapy); **and**
			  ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and**
			  ○ Authorization is for no more than 12 months

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Denosumab (continued)	Apr. 1, 2026		• **Reauthorization/Continuation of Care Criteria** ○ Documentation of positive clinical response to therapy; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Authorization is for no more than 12 months **Unproven** **Denosumab is unproven for the following indications:** • Combination therapy with intravenous bisphosphonates • Hyperparathyroidism • Immobilization hypercalcemia • Osteogenesis imperfecta • Osteopenia
FcRn Blockers	Apr. 1, 2026	**Title Change** • Previously titled *FcRn Blockers (Rystiggo®, Vyvgart®, & Vyvgart Hytrulo®)* **Coverage Rationale** • Removed reference link to the Medical Benefit Drug Policy titled *Review at Launch for New to Market Medications* for Imaavy™ (nipocalimab-aahu) • Revised list of applicable drug products for administration by a healthcare professional; added Imaavy (nipocalimab-aahu) for intravenous (IV) route ***Imaavy*** • Added language to indicate Imaavy is proven for the treatment of generalized myasthenia gravis in patients who are anti-acetylcholine receptor (AChR) antibody positive or antimuscle-specific tyrosine kinase (MuSK) antibody positive;	This policy refers to the following drug products for administration by a healthcare professional: • Imaavy (nipocalimab-aahu) for intravenous (IV) route • Rystiggo (rozanolixizumab-noli) for subcutaneous (SC) route • Vyvgart (efgartigimod alfa-fcab) for intravenous (IV) route • Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc) vial for subcutaneous (SC) route Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc) for self-administered subcutaneous injection is obtained under the pharmacy benefit, unless otherwise specified in the member's benefit plan documents. **Exception**: For members enrolled in UnitedHealthcare of California plans with a delegated provider group conducting the prior authorization review, the self-administered Vyvgart Hytrulo may be obtained under the medical benefit. **Imaavy is proven for the treatment of generalized myasthenia gravis in patients who are anti-acetylcholine receptor (AChR) antibody positive or antimuscle-specific tyrosine kinase (MuSK) antibody positive. Imaavy is medically necessary for the treatment of generalized myasthenia gravis in patients who are anti-AChR antibody positive or anti-MuSK antibody positive when all of the following criteria are met:**

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FcRn Blockers (continued)	Apr. 1, 2026	Imaavy is medically necessary for the treatment of generalized myasthenia gravis in patients who are anti-AChR antibody positive or anti-MuSK antibody positive when all of the following criteria are met: Initial Therapy ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming all of the following: ▪ Patient has not failed a previous course of Imaavy therapy ▪ Diagnosis of generalized myasthenia gravis (gMG) ▪ One of the following: – Positive serologic test for anti-AChR antibodies – Positive serologic test for anti-MuSK antibodies ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy ▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total score ≥ 5 at initiation of therapy ○ One of the following: ▪ If anti-acetylcholine receptor (AChR) antibody	• **Initial Therapy** ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming **all** of the following: ▪ Patient has not failed a previous course of Imaavy therapy; **and** ▪ Diagnosis of generalized myasthenia gravis (gMG); **and** ▪ **One** of the following: – Positive serologic test for anti-AChR antibodies; **or** – Positive serologic test for anti-MuSK antibodies **and** ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy; **and** ▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total score ≥ 5 at initiation of therapy **and** ○ **One** of the following: ▪ If anti-acetylcholine receptor (AChR) antibody positive, **one** of the following: – History of failure of at least **two** immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.); **or** – Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control **or** ▪ If anti-muscle-specific tyrosine kinase (MuSK) antibody positive: – History of failure of at least one immunosuppressive agent over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) **and** ○ Patient is **not** receiving Imaavy in combination with any of the following:

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FcRn Blockers (continued)	Apr. 1, 2026	positive, one of the following: – History of failure of at least two immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) – Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control ▪ If anti-muscle-specific tyrosine kinase (MuSK) antibody positive, history of failure of at least one immunosuppressive agent over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) ○ Patient is not receiving Imaavy in combination with any of the	▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ▪ An FcRn blocker [e.g., Rystiggo (rozanolixizumab-noli), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]; **and** ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) **and** ○ Imaavy is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Initial authorization will be for no more than 12 months • **Continuation of Therapy** ○ Patient has previously been treated with Imaavy; **and** ○ Submission of medical records (e.g., chart notes, laboratory tests) demonstrating **all** of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline; **and** ▪ Reduction in signs and symptoms of myasthenia gravis; **and** ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Imaavy (**Note**: Add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Imaavy therapy will be considered as treatment failure.) **and** ○ Patient is **not** receiving Imaavy in combination with any of the following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ▪ An FcRn blocker [e.g., Rystiggo (rozanolixizumab-noli), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]; **and**

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FcRn Blockers (continued)	Apr. 1, 2026	following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)] ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)] ▪ An FcRn blocker [e.g., Rystiggo (rozanolixizumab-noli), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)] ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) ○ Imaavy is dosed according to the U.S. FDA labeled dosing for gMG ○ Prescribed by, or in consultation with, a neurologist ○ Initial authorization will be for no more than 12 months Continuation of Therapy ○ Patient has previously been treated with Imaavy ○ Submission of medical records (e.g., chart notes, laboratory tests) demonstrating all of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline	▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) **and** ○ Imaavy is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Reauthorization will be for no more than 12 months **Rystiggo is proven for the treatment of generalized myasthenia gravis in patients who are anti-acetylcholine receptor (AChR) antibody positive or antimuscle-specific tyrosine kinase (MuSK) antibody positive. Rystiggo is medically necessary for the treatment of generalized myasthenia gravis in patients who are anti-AChR antibody positive or anti-MuSK antibody positive when all of the following criteria are met:** ● **Initial Therapy** ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming **all** of the following: ▪ Patient has not failed a previous course of Rystiggo therapy; **and** ▪ Diagnosis of generalized myasthenia gravis (gMG); **and** ▪ **One** of the following: – Positive serologic test for anti-AChR antibodies; **or** – Positive serologic test for anti-MuSK antibodies **and** ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy; **and** ▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total score ≥ 5 at initiation of therapy **and** ○ **One** of the following: ▪ If anti-acetylcholine receptor (AChR) antibody positive, one of the following: – History of failure of at least **two** immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.); **or**

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FcRn Blockers (continued)	Apr. 1, 2026	▪ Reduction in signs and symptoms of myasthenia gravis ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Imaavy (note: add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Imaavy therapy will be considered as treatment failure) ○ Patient is not receiving Imaavy in combination with any of the following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)] ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)] ▪ An FcRn blocker [e.g., Rystiggo (rozanolixizumab-noli), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)] ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard)	– Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control **or** ▪ If anti-muscle-specific tyrosine kinase (MuSK) antibody positive: – History of failure of at least one immunosuppressive agent over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) **and** ○ Patient is **not** receiving Rystiggo in combination with any of the following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ▪ An FcRn blocker [e.g., Imaavy (nipocalimab-aahu), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]; **and** ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) **and** ○ Patient will be given Rystiggo no sooner than 63 days from the start of the previous treatment cycle; **and** ○ Rystiggo is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Initial authorization will be for no more than 12 months • **Continuation of Therapy** ○ Patient has previously been treated with Rystiggo; **and** ○ Submission of medical records (e.g., chart notes, laboratory tests) demonstrating **all** of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline; **and**

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FcRn Blockers (continued)	Apr. 1, 2026	○ Imaavy is dosed according to the U.S. FDA labeled dosing for gMG ○ Prescribed by, or in consultation with, a neurologist ○ Reauthorization will be for no more than 12 months ***Rystiggo*** ● Revised list of drug products the patient must not be receiving in combination with Rystiggo: ○ Added "a CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]" ○ Replaced: ▪ "*Another* FcRn blocker [e.g., Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]" with "*an* FcRn blocker [e.g., *Imaavy (nipocalimab-aahu)*, Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]" ▪ "An immune globulin" with "an immune globulin *(e.g., Hizentra, Privigen, Gammagard)*" ***Vyvgart and Vyvgart Hytrulo for Generalized Myasthenia Gravis*** ● Revised list of drug products the patient must not be receiving in combination with Vyvgart or Vyvgart Hytrulo:	▪ Reduction in signs and symptoms of myasthenia gravis; **and** ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Rystiggo (**Note**: Add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Rystiggo therapy will be considered as treatment failure.) **and** ○ Patient is **not** receiving Rystiggo in combination with any of the following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ▪ An FcRn blocker [e.g., Imaavy (nipocalimab-aahu), Vyvgart (efgartigimod alfa-fcab), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase-qvfc)]; **and** ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) ○ Patient is not being given Rystiggo sooner than 63 days from the start of the previous treatment cycle; **and** ○ Rystiggo is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Reauthorization will be for no more than 12 months **Vyvgart and Vyvgart Hytrulo are proven for the treatment of generalized myasthenia gravis in patients who are anti-acetylcholine receptor (AChR) antibody positive. Vyvgart and Vyvgart Hytrulo are medically necessary for the treatment of generalized myasthenia gravis in patients who are anti-AChR antibody positive when all of the following criteria are met:** ● **Initial Therapy** ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming **all** of the following: ▪ Patient has not failed a previous course of Vyvgart therapy; **and** ▪ Patient has not failed a previous course of Vyvgart Hytrulo

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FcRn Blockers (continued)	Apr. 1, 2026	○ Added "a CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]" ○ Replaced: ▪ "*Another* FcRn blocker [e.g., Rystiggo (rozanolixizumab-noli)]" with "*an* FcRn blocker [e.g., *Imaavy (nipocalimab-aahu)*, Rystiggo (rozanolixizumab-noli)]" ▪ "An immune globulin" with "an immune globulin *(e.g., Hizentra, Privigen, Gammagard)*" ● Revised coverage criteria; removed criterion requiring the patient is not being given Vyvgart or Vyvgart Hytrulo sooner than 50 days from the start of the previous treatment cycle **Supporting Information** ● Updated *Background*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	therapy; **and** ▪ Diagnosis of generalized myasthenia gravis (gMG); **and** ▪ Positive serologic test for anti-AChR antibodies; **and** ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy; **and** ▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total score ≥ 5 at initiation of therapy **and** ○ **One** of the following: ▪ History of failure of at least **two** immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.); **or** ▪ Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control **and** ○ Patient is **not** receiving Vyvgart or Vyvgart Hytrulo in combination with any of the following: ○ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ○ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ○ An FcRn blocker [e.g., Imaavy (nipocalimab-aahu), Rystiggo (rozanolixizumab-noli)]; **and** ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) **and** ○ Vyvgart or Vyvgart Hytrulo is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Initial authorization will be for no more than 12 months ● **Continuation of Therapy** ○ Patient has previously been treated with Vyvgart or Vyvgart Hytrulo; **and**

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FcRn Blockers (continued)	Apr. 1, 2026		○ Submission of medical records (e.g., chart notes, laboratory tests) demonstrating **all** of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline; **and** ▪ Reduction in signs and symptoms of myasthenia gravis; **and** ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Vyvgart or Vyvgart Hytrulo (**Note**: Add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Vyvgart or Vyvgart Hytrulo therapy will be considered as treatment failure.) **and** ○ Patient is **not** receiving Vyvgart or Vyvgart Hytrulo in combination with any of the following: ▪ A CD19-directed cytolytic antibody [e.g., Uplizna (inebilizumab)]; **and** ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ▪ An FcRn blocker [e.g., Imaavy (nipocalimab-aahu), Rystiggo (rozanolixizumab-noli)]; **and** ▪ An immune globulin (e.g., Hizentra, Privigen, Gammagard) **and** ○ Vyvgart or Vyvgart Hytrulo is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Reauthorization will be for no more than 12 months **Vyvgart Hytrulo is proven for the treatment of chronic inflammatory demyelinating polyneuropathy (CIDP). Vyvgart Hytrulo is medically necessary for the treatment of CIDP when all of the following criteria are met:** ● **Initial Therapy** ○ Patient has not failed a previous course of Vyvgart Hytrulo therapy; **and** ○ Diagnosis of chronic inflammatory demyelinating polyneuropathy

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FcRn Blockers (continued)	Apr. 1, 2026		(CIDP) as confirmed by **all** of the following: ▪ Progressive symptoms present for at least 2 months; **and** ▪ Symptomatic polyradiculoneuropathy as indicated by progressive or relapsing motor or sensory impairment of more than one limb; **and** ▪ Electrodiagnostic findings (consistent with EFNS/PNS guidelines for definite CIDP) indicating the presence of at least **one** of the following: – Motor distal latency prolongation in 2 nerves – Reduction of motor conduction velocity in 2 nerves – Prolongation of F-wave latency in 2 nerves – Absence of F-waves in at least 1 nerve – Partial motor conduction block of at least 1 motor nerve – Abnormal temporal dispersion in at least 2 nerves – Distal CMAP duration increase in at least 1 nerve **and** ○ Trial and failure (after a trial of at least two months), contraindication, or intolerance to corticosteroids; **and** ○ **One** of the following: ▪ Trial and failure (after a trial of at least three months) to an immune globulin (i.e., intravenous immunoglobulin or subcutaneous immunoglobulin); **or** ▪ **Both** of the following: – Intolerance to all immune globulins (i.e., intravenous immunoglobulin or subcutaneous immunoglobulin); **and** – Dose has been adjusted or escalated to the maximally allowable and/or tolerated dose **or** ▪ Contraindication to **all** immune globulins (i.e., intravenous immunoglobulin or subcutaneous immunoglobulin) **and** ○ Patient is **not** receiving Vyvgart Hytrulo in combination with an immune globulin; **and** ○ Vyvgart Hytrulo is dosed according to the U.S. FDA labeled dosing for CIDP; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Initial authorization will be for no more than 12 months

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FcRn Blockers (continued)	Apr. 1, 2026		• **Continuation of Therapy** ○ Patient has previously been treated with Vyvgart Hytrulo; **and** ○ Documentation of positive clinical response to therapy as measured by an objective scale [e.g., Rankin, Modified Rankin, Medical Research Council (MRC) scale]; **and** ○ Patient is **not** receiving Vyvgart Hytrulo in combination with an immune globulin; **and** ○ Vyvgart Hytrulo is dosed according to the U.S. FDA labeled dosing for CIDP; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Reauthorization will be for no more than 12 months
Oncology Medication Clinical Coverage	Apr. 1, 2026	**Coverage Rationale** ***Oncology Products*** • Revised list of UnitedHealthcare preferred and non-preferred oncology products; added: All Oncology Indications ○ Beizray (HCPCS code J9174) (non-preferred) ○ Carboplatin (HCPCS code J9045) (preferred) ○ Jobevne (bevacizumab-nwgd) (non-preferred) ○ Kyxata (non-preferred) Non-Small Cell Lung Cancer ○ Keytruda Qlex (pembrolizumab and berahyaluronidase alfa-pmph) (preferred) Head and Neck Cancers ○ Keytruda Qlex (pembrolizumab and berahyaluronidase alfa-pmph) (non-preferred) Squamous Cell Skin Cancer ○ Keytruda (pembrolizumab) (non-preferred)	**Description** This policy provides parameters for coverage of injectable oncology medications (including, but not limited to octreotide acetate, leuprolide acetate, leucovorin, and levoleucovorin), including therapeutic radiopharmaceuticals, covered under the medical benefit based upon the National Comprehensive Cancer Network (NCCN) Drugs & Biologics Compendium® (NCCN Compendium®). The Compendium lists the appropriate drugs and biologics for specific cancers using US Food and Drug Administration (FDA)-approved disease indications and specific NCCN panel recommendations. Each recommendation is supported by a level of evidence category. Coverage of White Blood Cell Colony Stimulating Factors and Erythropoiesis-Stimulating Agents are addressed in separate policies. This policy does not provide coverage criteria for Chimeric Antigen Receptor (CAR)-T cell or Tumor-Infiltrating Lymphocyte (TIL) cell products. Coverage determinations are based on the member's benefits and the OptumHealth Transplant Solutions criteria for covered transplants; refer to the Clinical Guideline titled Chimeric Antigen Receptor T-cell (CAR T) Therapy, Tumor-Infiltrating Lymphocyte (TIL) Cell Therapy. **Coverage Rationale** ***Medical Necessity Plans*** The Oncology Products table in the policy lists the UnitedHealthcare preferred oncology products and respective non-preferred products. Coverage will be provided for the UnitedHealthcare preferred oncology

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oncology Medication Clinical Coverage (continued)	Apr. 1, 2026	○ Keytruda Qlex (pembrolizumab and berahyaluronidase alfa-pmph) (non-preferred) ○ Libtayo (cemiplimab-rwlc) (preferred) ○ Opdivo (nivolumab) (non-preferred) ○ Opdivo Qvantig (nivolumab and hyaluronidase-nvhy) (non-preferred) ○ Unloxcyt (cosibelimab-ipdl) (non-preferred) • Added language to clarify any U.S. FDA-approved product that may belong to UnitedHealthcare Preferred or Non-preferred Oncology Product categories *and/or are approved via FDA 505(b)(2) approval process* but not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare P&T committee ***Injectable Oncology Medications*** • Added language to indicate coverage determinations shall be based upon U.S. FDA labelling when there is no NCCN Drugs and Biologics Compendium with Categories of Evidence and Consensus **Applicable Codes** • Added HCPCS codes J9045, J9174, and J9275 **Supporting Information** • Updated *CMS* and *References* sections to reflect the most current	product contingent on the coverage criteria in the *Diagnosis-Specific Criteria* section of the policy. Coverage for any respective non-preferred oncology product will be provided contingent on the criteria in the *Preferred Product Criteria* and the *Diagnosis-Specific Criteria* sections of the policy. Members new to therapy will be required to utilize the UnitedHealthcare preferred oncology product unless they meet the criteria in this section. ***Preferred Product Criteria*** **Treatment with the respective non-preferred product specified in the Oncology Products table in the policy is medically necessary for oncology indications when both of the following are met:** • History of intolerance or contraindication to one of the UnitedHealthcare's preferred oncology products; and • Physician attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with the respective non-preferred product **Oncology Products** Refer to the policy for a list of UnitedHealthcare preferred and non-preferred oncology products and corresponding indications. Any U.S. Food and Drug Administration approved product that may belong to UnitedHealthcare Preferred or Non-preferred Oncology Product categories and/or are approved via FDA 505(b)(2) approval process but not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare P&T committee. **Diagnosis-Specific Criteria** ***Injectable Oncology Medications*** UnitedHealthcare recognizes indications and uses of injectable oncology medications, including therapeutic radiopharmaceuticals, listed in the NCCN Drugs and Biologics Compendium with Categories of Evidence and Consensus of 1, 2A, and 2B as proven and medically necessary, and Categories of Evidence and Consensus of 3 as unproven and not medically

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Oncology Medication Clinical Coverage (continued)	Apr. 1, 2026	information	necessary. When there is no NCCN Drugs and Biologics Compendium with Categories of Evidence and Consensus, coverage determinations shall be based upon FDA labelling UnitedHealthcare will cover all chemotherapy agents for individuals under the age of 19 years for oncology indications. The majority of pediatric patients receive treatments on national pediatric protocols that are quite similar in concept to the NCCN patient care guidelines. Refer to *Preferred Product Criteria* above for the UnitedHealthcare preferred oncology products and indications.
Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors	Apr. 1, 2026	**Related Policies** • Removed reference link to the Medical Benefit Drug Policy titled *Oncology Medication Clinical Coverage* **Coverage Rationale** • Added language to indicate: ○ Ahzantive™ (aflibercept-mrbb), Enzeevu™ (aflibercept-abzv), Eydenzelt® (aflibercept-boav), Nufymco® (ranibizumab-leyk), Opuviz™ (aflibercept-yszy), and Yesafili™ (aflibercept-jbvf) have been added to the Review at Launch program and some members may not be eligible for coverage of this medication at this time; refer to the Medical Benefit Drug Policy titled *Review at Launch for New to Market Medications* for additional details ○ Any U.S. FDA-approved ophthalmologic VEGF inhibitor product not listed by name in this policy will be considered	**Ahzantive™ (aflibercept-mrbb), Enzeevu™ (aflibercept-abzv), Eydenzelt® (aflibercept-boav), Nufymco® (ranibizumab-leyk), Opuviz™ (aflibercept-yszy), and Yesafili™ (aflibercept-jbvf) have been added to the Review at Launch program. Some members may not be eligible for coverage of this medication at this time. Refer to the Medical Benefit Drug Policy titled Review at Launch for New to Market Medications for additional details.** This policy provides information about the use of certain specialty pharmacy medications administered by the intravitreal route for ophthalmologic conditions. This policy refers to the following vascular endothelial growth factor (VEGF) inhibitors and dual VEGF/angiopoietin-2 (Ang-2) inhibitors: • Avastin® (bevacizumab) • Beovu® (brolucizumab-dbll) • Byooviz™ (ranibizumab-nuna) • Cimerli™ (ranibizumab-eqrn) • Eylea® (aflibercept) • Eylea® HD (aflibercept) • Lucentis® (ranibizumab) • Pavblu™ (aflibercept-ayyh) • Vabysmo™ (faricimab-svoa) • Any FDA-approved ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product not listed here*

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	non-preferred until reviewed by UnitedHealthcare • Revised list of applicable vascular endothelial growth factor (VEGF) inhibitors and dual VEGF/angiopoietin-2 (Ang-2) inhibitors; added "any U.S. FDA-approved ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product not listed [in this policy]" • Removed language indicating: ○ For requests that require medical necessity review, refer to the *General Requirements* and *Diagnosis-Specific Requirements* sections [of the policy] ○ Coverage for Avastin®, Cimerli™, Eylea®, Lucentis®, and Vabysmo™ is contingent on criteria in the *General Requirements* and *Diagnosis-Specific Requirements* sections [of the policy] ***Medical Necessity Plans*** • Added language to indicate: ○ Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo are the preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products; coverage will be provided for Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo contingent on the coverage criteria in the *General Requirements* and *Diagnosis-*	*Any U.S. Food and Drug Administration approved ophthalmologic VEGF inhibitor product not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare. **Beovu® and Byooviz™ are typically excluded from coverage. Coverage reviews may be in place if required by law or the benefit plan.** Refer to the Medical Benefit Drug Policy titled Medical Benefit Therapeutic Equivalent Medications – Excluded Drugs and the corresponding excluded drug list with preferred alternatives. **Medical Necessity Plans** Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo are the preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products. Coverage will be provided for Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo contingent on the coverage criteria in the General Requirements and Diagnosis-Specific Criteria sections. Coverage for Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products will be provided contingent on the criteria in the Preferred Product Criteria, General Requirements and Diagnosis-Specific Criteria sections. In order to continue coverage, members already on Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products will be required to change therapy to Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pabvlu or Vabysmo unless they meet the criteria in the Preferred Product Criteria section. ***Preferred Product Criteria*** **Treatment with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products is medically necessary for the indications specified in the policy when one of the following is met:** • **Both** of the following: ○ History of a trial of adequate dose and duration of Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo, resulting in minimal clinical response; **and**

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	*Specific Criteria* sections [of the policy] ○ Coverage for Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products will be provided contingent on the criteria in the *Preferred Product Criteria*, *General Requirements*, and *Diagnosis-Specific Criteria* sections [of the policy] ○ In order to continue coverage, members already on Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products will be required to change therapy to Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, or Vabysmo unless they meet the criteria in the *Preferred Product Criteria* section [of the policy] Preferred Product Criteria ○ Treatment with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor products is medically necessary for the indications specified in the policy when one of the following is met: ▪ Both of the following: – History of a trial of adequate dose and duration of Avastin,	○ Physician attests that, in their clinical opinion, the clinical response would be expected to be superior with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product, than experienced with Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo **or** • **Both** of the following: ○ History of intolerance, contraindication, or adverse event to Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo; **and** ○ Physician attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product **Non-Medical Necessity Plans** Any ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product is to be approved contingent on the coverage criteria in the General Requirements and Diagnosis-Specific Criteria sections. ***General Requirements (Applicable to all Requests)*** • For **initial therapy**, **all** of the following: ○ Diagnosis; **and** ○ Intravitreal VEGF or dual VEGF/Ang-2 inhibitor administration is no more than 12 doses per year per eye, regardless of diagnosis; **and** ○ Authorization is for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response to anti-VEGF therapy; **and** ○ Intravitreal VEGF or dual VEGF/Ang-2 inhibitor administration is no more than 12 doses per year per eye, regardless of diagnosis; **and** ○ Authorization is for no more than 12 months ***Diagnosis-Specific Requirements (Applicable to all Requests)*** **Avastin (bevacizumab) is proven and medically necessary for the treatment of:**

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo, resulting in minimal clinical response – Physician attests that, in their clinical opinion, the clinical response would be expected to be superior with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product, than experienced with Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo ▪ Both of the following: – History of intolerance, contraindication, or adverse event to Avastin, Cimerli, Eylea, Eylea HD, Lucentis, Pavblu, and Vabysmo – Physician attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with Beovu, Byooviz, or other non-preferred ophthalmologic VEGF	• Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Myopic choroidal neovascularization (mCNV) • Neovascular age-related macular degeneration (nAMD) • Neovascular glaucoma (NVG) • Neovascularization of the iris (NVI) • Type I Retinopathy of Prematurity (ROP) **Beovu (brolucizumab) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Neovascular age-related macular degeneration (nAMD) **Byooviz (ranibizumab-nuna) is proven and medically necessary for the treatment of:** • Macular edema following retinal vein occlusion (RVO) • Myopic choroidal neovascularization (mCNV) • Neovascular age-related macular degeneration (nAMD) **Cimerli (ranibizumab-eqrn) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Myopic choroidal neovascularization (mCNV) • Neovascular age-related macular degeneration (nAMD) **Eylea (aflibercept) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Neovascular age-related macular degeneration (nAMD) • Retinopathy of prematurity (ROP)

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	or dual VEGF/Ang-2 inhibitor product ***Non-Medical Necessity Plans*** • Added language to indicate any ophthalmologic VEGF or dual VEGF/Ang-2 inhibitor product is to be approved contingent on the coverage criteria in the *General Requirements* and *Diagnosis-Specific Criteria* sections [of the policy] ***General Requirements*** • Replaced language indicating "[this section of the policy is] applicable to all *medical necessity* requests" with "[this section of the policy is] applicable to all requests" • Added language to indicate authorization is for no more than 12 months ***Diagnosis-Specific Requirements*** • Added language to indicate this section of the policy applies to all requests • Revised list of proven and medically necessary indications for: Avastin (bevacizumab) ○ Added: ▪ Diabetic retinopathy (DR) ▪ Myopic choroidal neovascularization (mCNV) ○ Removed: ▪ Choroidal neovascularization secondary to pathologic myopia, angioid	**Eylea HD (aflibercept) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Neovascular age-related macular degeneration (nAMD) **Lucentis (ranibizumab) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Myopic choroidal neovascularization (mCNV) • Neovascular age-related macular degeneration (nAMD) **Pavblu (aflibercept-ayyh) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Diabetic retinopathy (DR) • Macular edema following retinal vein occlusion (RVO) • Neovascular age-related macular degeneration (nAMD) **Vabysmo (faricimab-svoa) is proven and medically necessary for the treatment of:** • Diabetic macular edema (DME) • Macular edema following retinal vein occlusion (RVO) • Neovascular age-related macular degeneration (nAMD) **Additional Information** Avastin (bevacizumab) is supplied in sterile vials containing a solution of 25 mg/mL. Doses utilized in ophthalmic conditions generally range from 6.2 mcg to 2.5 mg. Therefore, bevacizumab vials are often divided into single-dose, prefilled syringes for intravitreal use by compounding pharmacies. Compounding pharmacies must comply with the United States Pharmacopeia (USP) General Chapter < 797 > : Pharmaceutical Compounding – Sterile Preparations, which sets standards for the

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	streaks/pseudoxanthoma elasticum, or ocular histoplasmosis syndrome (OHS) ▪ Proliferative diabetic retinopathy ○ Replaced: ▪ "Macular edema *secondary to branch retinal vein occlusion (BRVO) or central* retinal vein occlusion (*CRVO*)" with "macular edema *following* retinal vein occlusion (*RVO*)" ▪ "Neovascularization of the iris (NVI) *(rubeosis iridis)*" with "neovascularization of the iris (NVI)" Eylea (aflibercept) ○ Replaced "macular edema *secondary to branch retinal vein occlusion (BRVO) or central* retinal vein occlusion (*CRVO*)" with "macular edema *following* retinal vein occlusion (*RVO*)" Eylea HD (aflibercept) ○ Added "macular edema following retinal vein occlusion (RVO)" Lucentis (ranibizumab) ○ Added "myopic choroidal neovascularization (mCNV)" ○ Removed "choroidal neovascularization secondary to pathologic myopia, angioid streaks/pseudoxanthoma	compounding, transportation, and storage of compounded sterile products (CSP). The American Academy of Opthalmology recommends sourcing bevacizumab for intravitreal injections from a compounding pharmacy accredited by the Pharmacy Compounding Accrediation Board (PCAB), which adheres to these standards.

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	elasticum, or ocular histoplasmosis syndrome (OHS)" ○ Replaced "macular edema *secondary to branch retinal vein occlusion (BRVO) or central* retinal vein occlusion (*CRVO*)" with "macular edema *following* retinal vein occlusion (*RVO*)" ***Additional Information*** ● Revised language to indicate: ○ Avastin (bevacizumab) is supplied in sterile vials containing a solution of 25 mg/mL and doses utilized in ophthalmic conditions generally range from 6.2 mcg to 2.5 mg; therefore, bevacizumab vials are often divided into single-dose, prefilled syringes for intravitreal use by compounding pharmacies ○ Compounding pharmacies must comply with the United States Pharmacopeia (USP) General Chapter 797: *Pharmaceutical Compounding – Sterile Preparations*, which sets standards for the compounding, transportation, and storage of compounded sterile products (CSP) ○ The American Academy of Ophthalmology recommends sourcing bevacizumab for intravitreal injections from a compounding pharmacy	

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Ophthalmologic Vascular Endothelial Growth Factor (VEGF) Inhibitors (continued)	Apr. 1, 2026	accredited by the Pharmacy Compounding Accreditation Board (PCAB), which adheres to these standards **Definitions** • Removed definition of "Type I Retinopathy of Prematurity (ROP)" **Applicable Codes** • Added HCPCS code C9257 • Updated list of applicable ICD-10 diagnosis codes for HCPCS code J0177; added H34.8110, H34.8111, H34.8112, H34.8120, H34.8121, H34.8122, H34.8130, H34.8131, H34.8132, H34.8190, H34.8191, H34.8192, H34.821, H34.822, H34.823, H34.829, H34.8310, H34.8311, H34.8312, H34.8320, H34.8321, H34.8322, H34.8330, H34.8331, H34.8332, H34.8390, H34.8391, and H34.8392 **Supporting Information** • Updated *Background*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	
Provider Administered Drugs – Site of Care	Apr. 1, 2026	**Coverage Rationale** • Revised coverage criteria: ○ Added criterion to allow coverage when the patient is medically unstable and is at risk of requiring medical services and equipment available only in an outpatient hospital setting (e.g., endotracheal tube, chest tube	This policy addresses the criteria for consideration of allowing hospital outpatient facility infusion services for specialty medications and intravenous Immune Globulin (IVIG) and subcutaneous Immune Globulin (SCIG) therapy. This includes claim submission for hospital-based services with the following CMS/AMA place of service codes: • 19 Off Campus-Outpatient Hospital; and • 22 On Campus-Outpatient Hospital Alternative Sites of Care, such as non-hospital outpatient infusion, physician office, ambulatory infusion suites, or home infusion services are

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Provider Administered Drugs – Site of Care (continued)	Apr. 1, 2026	insertion equipment, cricothyroidotomy set, mechanical ventilator) during administration of the requested drug based on risk of becoming medically unstable due to concomitant use of medications or other drug-drug or drug-disease interaction(s) ○ Replaced criterion requiring "[the patient has] severe patent vascular access issues (for intravenous infusions only) that require specialized equipment only available in an outpatient hospital setting (e.g., ultrasound guidance) *and is not a viable candidate for long-term vascular access devices such as picc line or port-a-cath*" with "[the patient has] severe patent vascular access issues (for intravenous infusions only) that require specialized equipment only available in an outpatient hospital setting (e.g., ultrasound guidance)" • Revised list of medications that require healthcare provider administration; added Imaavy™ (nipocalimab-aahu) **Documentation Requirements** • Revised list of specialty medications with associated documentation requirements; added Imaavy™ (nipocalimab-aahu)	well accepted places of service for medication infusion therapy. If an individual does not meet criteria for outpatient hospital facility infusion, alternative sites of care may be used. **Submission of medical records documenting that outpatient hospital facility-based administration is medically necessary for individuals who meet at least one of the following criteria:** • The patient is medically unstable and is at risk of requiring medical services and equipment available only in an outpatient hospital setting (e.g., endotracheal tube, chest tube insertion equipment, cricothyroidotomy set, mechanical ventilator) during administration of the requested drug based on **one** of the following: ○ History of cardiopulmonary conditions that cause an increased risk of severe adverse reactions during or immediately following infusion; **or** ○ An inability to tolerate fluid volume load (for intravenous infusions only) despite using the minimum amount of fluid required for infusion (e.g., unstable renal function); **or** ○ Risk of becoming medically unstable due to concomitant use of medications or other drug-drug or drug-disease interaction(s) **or** • Treatment at an alternative Site of Care presents a health risk due to a clinically significant physical or cognitive impairment; **or** • Severe patent vascular access issues (for intravenous infusions only) that require specialized equipment only available in an outpatient hospital setting (e.g., ultrasound guidance); **or** • Previous episode(s) of severe or potentially life-threatening adverse events (e.g., anaphylaxis, seizure, thromboembolism, myocardial infarction, renal failure), not including the first or second infusion, that have occurred while receiving requested therapy that was unresponsive to acetaminophen, steroids, diphenhydramine, fluids, infusion rate reductions, or other pre-medications, thereby increasing risk to the individual while administering at alternative Sites of Care; **or** • Initial infusion or re-initiation of previous therapy after more than 6 months (excludes drugs dosed at an interval of 6 months or greater) for a short duration of time (e.g., 4 weeks); **or**

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Provider Administered Drugs – Site of Care (continued)	Apr. 1, 2026	**Applicable Codes**
		• Added HCPCS code J9256
			• **For IVIG or SCIG only**: Individual has immunoglobulin A (IgA) deficiency with anti-IgA antibodies; **or**
			• **All** of the following:
			  ○ Homecare or home infusion provider has deemed that the individual or home environment is not suitable for home infusion therapy; **and**
			  ○ The prescriber is unable to administer in the office setting; **and**
			  ○ There are no ambulatory infusion suite options available for this member
			
			Ongoing outpatient hospital facility-based infusion duration of therapy will be no more than 6 months to allow for reassessment of the individual's ability to receive therapy at an alternative Site of Care.
			
			Note: If more than one of the above criteria are met, then the greatest of the applicable approval time periods will be allowed.

Somatostatin Analogs	Apr. 1, 2026	**Template Update**
		• Transferred content to shared policy template that applies to both UnitedHealthcare Commercial and Individual Exchange benefit plans
		• Added *Application* section to indicate this policy applies to:
		  ○ UnitedHealthcare Commercial benefit plans
		  ○ Individual Exchange benefit plans
		Coverage Rationale
		• Added language to indicate this policy refers to the following somatostatin analogs for non-oncology indications:
		  ○ Lanreotide® injection
		  ○ Sandostatin® (octreotide acetate)
		  ○ Sandostatin LAR® (octreotide acetate)
			This policy refers to the following somatostatin analogs for non-oncology indications:
			• Lanreotide® Injection
			• Sandostatin® (octreotide acetate)
			• Sandostatin LAR® (octreotide acetate)
			• Signifor LAR® (pasireotide)
			• Somatostatin Depot® (lanreotide)
			
			For oncology indications, refer to the Medical Benefit Drug Policy titled Oncology Medication Clinical Coverage for updated information based on the National Comprehensive Cancer Network (NCCN) Drugs & Biologics Compendium® (NCCN Compendium®).
			
			Sandostatin (octreotide acetate) is proven for the following conditions:
			• Acromegaly patients who have had inadequate response to or cannot be treated with surgical resection, pituitary irradiation, or bromocriptine mesylate at maximally tolerated doses
			• Severe diarrhea and flushing episodes associated with metastatic carcinoid tumors

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Somatostatin Analogs (continued)	Apr. 1, 2026	○ Signifor LAR® (pasireotide) ○ Somatostatin Depot® (lanreotide) • Removed language indicating: ○ Signifor is proven for the treatment of patients with Cushing's disease for whom pituitary surgery is not an option or has not been curative ○ Somatostatin analogs are unproven and not medically necessary for treating the following conditions: ▪ HIV-AIDS-related diarrhea ▪ Chylothorax ▪ Dumping syndrome ▪ Pancreatitis ▪ Persistent hyperinsulinemic hypoglycemia of infancy ▪ Prevention of postoperative complications following pancreatic surgery ▪ Short bowel syndrome ○ Somatostatin analogs are unproven for treating other conditions not listed [in the policy] as proven due to the lack of published clinical evidence of safety and/or efficacy in published peer-reviewed medical literature ○ In patients with carcinoid syndrome and vasoactive intestinal peptide tumors (VIPomas), the effect of Sandostatin and Sandostatin LAR on tumor size, rate of	• Profuse watery diarrhea associated with vasoactive intestinal peptide (VIP) secreting tumors **Sandostatin LAR (octreotide acetate LAR) is proven for the treatment of the following conditions:** • Acromegaly patients who have had inadequate response to or cannot be treated with surgery and/or radiotherapy • Severe diarrhea and flushing episodes associated with metastatic carcinoid tumors • Profuse watery diarrhea associated with vasoactive intestinal peptide (VIP) secreting tumors • Chemotherapy and/or radiation-induced diarrhea • Malignant bowel disease **Signifor LAR (pasireotide) is proven for the treatment of the following conditions:** • Acromegaly patients who have had an inadequate response to surgery or for whom surgery is not an option • Cushing's disease in patients for whom pituitary surgery is not an option or has not been curative **Somatuline Depot (lanreotide) and Lanreotide Injection are proven for the treatment of acromegaly in patients who have had an inadequate response to surgery and/or radiotherapy, or for who, surgery and/or radiotherapy is not an option.**

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Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Somatostatin Analogs (continued)	Apr. 1, 2026	growth, and development of metastases has not been determined • Revised coverage criteria for: ***Sandostatin (octreotide acetate)*** ○ Removed criterion requiring: ▪ Bleeding gastroesphoageal varices associated with liver disease ▪ Chemotherapy and/or radiation-induced diarrhea ▪ Malignant bowel disease ○ Replaced criterion requiring "acromegaly patients who have had inadequate response to or cannot be treated with surgical resection, pituitary irradiation, or *dopamine agonist (e.g., bromocriptine, cabergoline) therapy*" with "acromegaly patients who have had inadequate response to or cannot be treated with surgical resection, pituitary irradiation, or *bromocriptine mesylate at maximally tolerated doses*" ***Sandostatin LAR (octreotide acetate LAR)*** ○ Removed criterion requiring bleeding gastroesphoageal varices associated with liver disease ○ Replaced criterion requiring "acromegaly patients who have had inadequate response to or cannot be treated with *surgical*	

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Somatostatin Analogs (continued)	Apr. 1, 2026	*resection, pituitary irradiation, or dopamine agonist (e.g., bromocriptine, cabergoline) therapy*" with "acromegaly patients who have had inadequate response to or cannot be treated with *surgery and/or radiotherapy*" **Applicable Codes** • Added ICD-10 diagnosis codes C7A.023, C25.3, E24.8, and E24.9 • Removed ICD-10 diagnosis codes I85.01 and I85.11 **Supporting Information** • Updated *Background*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	
Uplizna® (Inebilizumab-Cdon)	Apr. 1, 2026	**Coverage Rationale** ***Neuromyelitis Optica Spectrum Disorder (NMOSD)*** • Revised coverage criteria; removed criterion requiring one of the following: ○ History of one or more relapses that required rescue therapy during the previous 12 months prior to initiating Uplizna ○ History of two or more relapses that required rescue therapy during the previous 24 months prior to initiating Uplizna ***Generalized Myasthenia Gravis (gMG)*** • Added language to indicate Uplizna is proven for the treatment of gMG	**Uplizna (inebilizumab-cdon) is proven for the treatment of neuromyelitis optica spectrum disorder (NMOSD). Uplizna (inebilizumab-cdon) is medically necessary for the treatment of neuromyelitis optica spectrum disorder (NMOSD) when all the following criteria are met:** • For **initial therapy**, **all** of the following: ○ Diagnosis of neuromyelitis optica spectrum disorder (NMOSD) by a neurologist confirming **all** of the following: ▪ Past medical history of **one** of the following: – Optic neuritis – Acute myelitis – Area postrema syndrome: episode of otherwise unexplained hiccups or nausea and vomiting – Acute brainstem syndrome – Symptomatic narcolepsy or acute diencephalic clinical syndrome with NMOSD-typical diencephalic MRI lesions – Symptomatic cerebral syndrome with NMOSD-typical brain lesions **and**

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Uplizna® (Inebilizumab-Cdon) (continued)	Apr. 1, 2026	in patients who are anti-acetylcholine receptor (AChR) antibody positive or antimuscle-specific tyrosine kinase (MuSK) antibody positive; Uplizna is medically necessary for the treatment of gMG in patients who are anti-AChR antibody positive or anti-MuSK antibody positive when all of the following criteria are met: Initial Therapy ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming all of the following: ▪ Patient has not failed a previous course of Uplizna therapy for the treatment of generalized myasthenia gravis ▪ Diagnosis of gMG ▪ One of the following: – Positive serologic test for anti-AChR antibodies – Positive serologic test for anti-MuSK antibodies ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy ▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total	▪ Positive serologic test for anti-aquaporin-4 immunoglobulin G (AQP4-IgG)/NMO-IgG antibodies; **and** ▪ Diagnosis of multiple sclerosis or other diagnoses have been ruled out **and** ○ **One** of the following: ▪ History of failure of rituximab therapy; **or** ▪ **Both** of the following: – History of intolerance or contraindication to rituximab; **and** – Physician attests that, in their clinical opinion, the same intolerance or severe adverse event would not be expected to occur with Uplizna **and** ○ Uplizna is initiated according to the U.S. FDA labeled dosing for NMOSD; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Patient is **not** receiving Uplizna in combination with **any** of the following for treatment of the same indication: ▪ Multiple sclerosis disease modifying therapies [e.g., dimethyl fumarate, fingolimod, Ocrevus (ocrelizumab), etc.] ▪ Complement inhibitors [e.g., eculizumab, PiaSky (crovalimab), Ultomiris (ravulizumab)] ▪ Anti-IL6 therapy (e.g., tocilizumab) ▪ Anti-CD20 therapy (e.g., rituximab) **and** ○ Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response; **and** ○ Uplizna is dosed according to the U.S. FDA labeled dosing for NMOSD; **and** ○ Patient is **not** receiving Uplizna in combination with **any** of the following for treatment of the same indication: ▪ Multiple sclerosis disease modifying therapies [e.g., dimethyl fumarate, fingolimod, Ocrevus (ocrelizumab), etc.] ▪ Anti-IL6 therapy (e.g., tocilizumab) ▪ Complement inhibitors [e.g., eculizumab, PiaSky (crovalimab), Ultomiris (ravulizumab)]

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Uplizna® (Inebilizumab-Cdon) (continued)	Apr. 1, 2026	score ≥ 5 at initiation of therapy ○ One of the following: ▪ If anti-acetylcholine receptor (AChR) antibody positive, one of the following: – History of failure of at least two immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) – Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control ▪ If anti-muscle-specific tyrosine kinase (MuSK) antibody positive, history of failure of at least one immunosuppressive agent over the course of at least 12 months (e.g., azathioprine, corticosteroids,	▪ Anti-CD20 therapy (e.g., rituximab) **and** ○ Reauthorization will be for no more than 12 months **Uplizna (inebilizumab-cdon) is proven for the treatment of Immunoglobulin G4-related disease (IgG4-RD). Uplizna (inebilizumab-cdon) is medically necessary for the treatment of Immunoglobulin G4-related disease (IgG4-RD) when all the following criteria are met:** ● For **initial therapy**, **all** of the following: ○ Diagnosis of Immunoglobulin G4-related disease (IgG4-RD); **and** ○ Confirmation of IgG4-RD by a positive assessment using the ACR/EULAR classification criteria, demonstrated by **all** of the following: ▪ Involvement of at least 1 or more organ(s) in a manner consistent with IgG4-RD; **and** ▪ Exclusion criteria is negative and consistent with an IgG4-RD diagnosis (e.g., clinical findings, serologic results, radiology assessments, pathology interpretations); **and** ▪ Inclusion criteria is positive and signifies a diagnosis of IgG4-RD (e.g., clinical findings, serologic results, radiology assessments, pathology interpretations) **and** ○ **Both** of the following: ▪ History of failure, contraindication, or intolerance to glucocorticoids; **and** ▪ **One** of the following: – History of failure of rituximab therapy; **or** – **Both** of the following: • History of intolerance or contraindication to rituximab; **and** • Physician attests that, in their clinical opinion, the same intolerance or severe adverse event would not be expected to occur with Uplizna **and** ○ Uplizna is initiated according to the U.S. FDA labeled dosing for IgG4-RD; **and**

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Uplizna® (Inebilizumab-Cdon) (continued)	Apr. 1, 2026	cyclosporine, methotrexate, mycophenolate, etc.) ○ Patient is not receiving Uplizna in combination with any of the following for treatment of the same indication: ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)] ▪ An FcRn blocker [e.g., Imaavy (nipocalimab), Rystiggo (rozanolixizumab), Vyvgart (efgartigimod alfa), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase)] ▪ An immune globulin (e.g., Hizentra, Gammagard, Alyglo, etc.) ○ Patient will be given Uplizna no sooner than two weeks after the first initial dose and every 6 months thereafter for subsequent infusions ○ Uplizna is dosed according to the U.S. FDA labeled dosing for gMG ○ Prescribed by, or in consultation with, a neurologist ○ Initial authorization will be for no more than 12 months Continuation of Therapy ○ Patient has previously been treated with Uplizna ○ Submission of medical records (e.g., chart notes, laboratory	○ Prescribed by, or in consultation with, a specialist with expertise in the treatment of IgG4-RD; **and** ○ Patient is **not** receiving Uplizna in combination with a disease modifying therapy for the treatment of IgG4-related disease (e.g., rituximab); **and** ○ Initial authorization will be for no more than 12 months ● For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response; **and** ○ Uplizna is dosed according to the U.S. FDA labeled dosing for IgG4-RD; **and** ○ Prescribed by, or in consultation with, a specialist with expertise in the treatment of IgG4-RD; **and** ○ Patient is **not** receiving Uplizna in combination with a disease modifying therapy for the treatment of IgG4-related disease (e.g., rituximab); **and** ○ Reauthorization will be for no more than 12 months **Uplizna is proven for the treatment of generalized myasthenia gravis who are anti-acetylcholine receptor (AChR) antibody positive or anti muscle-specific tyrosine kinase (MuSK) antibody positive. Uplizna is medically necessary for the treatment of generalized myasthenia gravis in patients who are anti-AChR antibody positive or anti-MuSK antibody positive when all of the following criteria are met:** ● For **initial therapy**, **all** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values, etc.) confirming **all** of the following: ▪ Patient has not failed a previous course of Uplizna therapy for the treatment of generalized myasthenia gravis; **and** ▪ Diagnosis of generalized myasthenia gravis (gMG); **and** ▪ **One** of the following: – Positive serologic test for anti-AChR antibodies; **or** – Positive serologic test for anti-MuSK antibodies **and** ▪ Patient has a Myasthenia Gravis Foundation of America (MGFA) Clinical Classification of class II, III, or IV at initiation of therapy; **and**

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Uplizna® (Inebilizumab-Cdon) (continued)	Apr. 1, 2026	tests) demonstrating all of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline ▪ Reduction in signs and symptoms of myasthenia gravis ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Uplizna (note: add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Uplizna therapy will be considered as treatment failure) ○ Patient is not receiving Uplizna in combination with any of the following for treatment of the same indication: ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)] ▪ An FcRn blocker [e.g., Imaavy (nipocalimab), Rystiggo (rozanolixizumab), Vyvgart	▪ Patient has a Myasthenia Gravis Activities of Daily Living scale (MG-ADL) total score ≥ 5 at initiation of therapy **and** ○ **One** of the following: ▪ If anti-acetylcholine receptor (AChR) antibody positive, one of the following: – History of failure of at least **two** immunosuppressive agents over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.); **or** – Patient has a history of failure of at least one immunosuppressive therapy and has required four or more courses of plasmapheresis/plasma exchanges and/or immune globulin over the course of at least 12 months without symptom control **or** ▪ If anti-muscle-specific tyrosine kinase (MuSK) antibody positive: – History of failure of at least one immunosuppressive agent over the course of at least 12 months (e.g., azathioprine, corticosteroids, cyclosporine, methotrexate, mycophenolate, etc.) **and** ○ Patient is **not** receiving Uplizna in combination with any of the following for treatment of the same indication: ○ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ○ An FcRn blocker [e.g., Imaavy (nipocalimab), Rystiggo (rozanolixizumab), Vyvgart (efgartigimod alfa), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase)]; **and** ○ An immune globulin (e.g., Hizentra, Gammagard, Alyglo, etc.) **and** ○ Patient will be given Uplizna no sooner than two weeks after the first initial dose and every 6 months thereafter for subsequent infusions; **and** ○ Uplizna is dosed according to the U.S. FDA labeled dosing for gMG; **and**

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Uplizna® (Inebilizumab-Cdon) (continued)	Apr. 1, 2026	(efgartigimod alfa), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase)] ▪ An immune globulin (e.g., Hizentra, Gammagard, Alyglo, etc.) ○ Patient is not being given Uplizna sooner than 6 months from the start of the previous treatment cycle ○ Uplizna is dosed according to the U.S. FDA labeled dosing for gMG ○ Prescribed by, or in consultation with, a neurologist ○ Reauthorization will be for no more than 12 months **Applicable Codes** • Added ICD-10 diagnosis codes G70.00 and G70.01 **Supporting Information** • Updated *Background*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	○ Prescribed by, or in consultation with, a neurologist; **and** ○ Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Patient has previously been treated with Uplizna; **and** ○ Submission of medical records (e.g., chart notes, laboratory tests) demonstrating **all** of the following: ▪ Improvement and/or maintenance of at least a 2 point improvement (reduction in score) in the MG-ADL score from pre-treatment baseline; **and** ▪ Reduction in signs and symptoms of myasthenia gravis; **and** ▪ Maintenance, reduction, or discontinuation of dose(s) of baseline immunosuppressive therapy (IST) prior to starting Uplizna (**Note**: Add on, dose escalation of IST, or additional rescue therapy from baseline to treat myasthenia gravis or exacerbation of symptoms while on Uplizna therapy will be considered as treatment failure.) **and** ○ Patient is **not** receiving Uplizna in combination with any of the following for treatment of the same indication: ▪ A complement inhibitor [e.g., eculizumab, Ultomiris (ravulizumab), Zilbrysq (zilucoplan)]; **and** ○ An FcRn blocker [e.g., Imaavy (nipocalimab), Rystiggo (rozanolixizumab), Vyvgart (efgartigimod alfa), Vyvgart Hytrulo (efgartigimod alfa and hyaluronidase)]; **and** ▪ An immune globulin (e.g., Hizentra, Gammagard, Alyglo, etc.) **and** ○ Patient is not being given Uplizna sooner than 6 months from the start of the previous treatment cycle; **and** ○ Uplizna is dosed according to the U.S. FDA labeled dosing for gMG; **and** ○ Prescribed by, or in consultation with, a neurologist; **and** ○ Reauthorization will be for no more than 12 months

General Information

The inclusion of a health service (e.g., test, drug, device, or procedure) in this bulletin indicates only that UnitedHealthcare is adopting a new policy and/or updated, revised, replaced, or retired an existing policy; it does not imply that UnitedHealthcare provides coverage for the health service. Note that most benefit plan documents exclude from benefit coverage health services identified as investigational or unproven/not medically necessary. Physicians and other health care professionals may not seek or collect payment from a member for services not covered by the applicable benefit plan unless first obtaining the member's written consent, acknowledging that the service is not covered by the benefit plan and that they will be billed directly for the service.

Note: The absence of a policy does not automatically indicate or imply coverage. As always, coverage for a health service must be determined in accordance with the member's benefit plan and any applicable federal or state regulatory requirements. Additionally, UnitedHealthcare reserves the right to review the clinical evidence supporting the safety and effectiveness of a medical technology prior to rendering a coverage determination.

UnitedHealthcare respects the expertise of the physicians, health care professionals, and their staff who participate in our network. Our goal is to support you and your patients in making the most informed decisions regarding the choice of quality and cost-effective care, and to support practice staff with a simple and predictable administrative experience. The Medical Policy Update Bulletin was developed to share important information regarding changes to our Medical Policies and Medical Benefit Drug Policies. When information in this bulletin conflicts with applicable state and/or federal law, UnitedHealthcare follows such applicable federal and/or state law.

Policy Update Classifications

New

New clinical coverage criteria have been adopted for a health service (e.g., test, drug, device, or procedure)

Updated

An existing policy has been reviewed and changes have not been made to the clinical coverage criteria; however, items such as the clinical evidence, FDA information, and/or list(s) of applicable codes may have been updated

Revised

An existing policy has been reviewed and revisions have been made to the clinical coverage criteria

Replaced

An existing policy has been replaced with a new or different policy

Retired

The health service(s) addressed in the policy are no longer being managed or are considered to be proven/medically necessary and are therefore not excluded as unproven/not medically necessary services, unless coverage guidelines or criteria are otherwise documented in another policy

The complete library of UnitedHealthcare Medical Policies and Medical Benefit Drug Policies is available at **UHCprovider.com/policies** > For Commercial Plans > Medical & Drug Policies.